

Dynamic Guideline: WHO-Based Hepatitis - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Index WHO hepatitis guidance used for this dynamic report: WHO’s **15 February 2026** consolidated implementation handbook is the most current WHO hepatitis B/C/D-related guidance identified for prevention, testing, treatment, service delivery and monitoring. WHO describes it as the first consolidated handbook bringing together WHO recommendations on hepatitis B and C and including hepatitis D testing strategies, drawing on evidence-based guidance issued from 2015–2025 [WHO consolidated handbook](#). WHO also announced the handbook on **10 March 2026**, stating that it consolidates more than 80 evidence-informed recommendations on hepatitis B, C and D [WHO news update](#).

Exactly 5 WHO hepatitis-related guidelines/guidance documents synthesized

No.	WHO guideline/guidance title	Publication date	Target population	Scope and objectives	GRADE / evidence note
1	Consolidated guidance on hepatitis B and C prevention, testing, treatment, service delivery and monitoring: an implementation handbook for a public health approach	15 February 2026	People at risk of or living with HBV, HCV and HDV; programme managers; clinicians; policymakers	Consolidates WHO recommendations on HBV/HCV prevention, testing, treatment, monitoring and service delivery; includes HDV testing strategies; promotes simplified, person-centred public-health approaches WHO consolidated handbook .	WHO describes recommendations as evidence-informed and drawn from 2015–2025 guidance WHO news update .
2	Guidelines for the prevention, diagnosis, care and treatment for people with chronic hepatitis B infection	29 March 2024	Adults, adolescents, pregnant women and people living with chronic HBV	Updates treatment eligibility, pregnancy prophylaxis, HBV DNA testing, reflex testing, HDV testing, adherence and service delivery WHO HBV guideline .	WHO provides evidence-to-decision and GRADE tables, systematic reviews, modelling, landscape reports, COI declarations and values/preferences annexes WHO HBV guideline .
3	Consolidated guidelines on person-centred viral hepatitis strategic information: using data to support country scale-up of hepatitis prevention, diagnosis and treatment services	9 April 2024	National and subnational hepatitis programmes; health-information systems	Provides WHO’s approach to collect, analyse, report, disseminate and use viral hepatitis strategic information; emphasizes person-centred monitoring and stepwise surveillance strengthening WHO strategic information guideline .	GRADE use was not directly verifiable from the available snippets; recommendations are treated here as public-health programme recommendations with indirect evidence.
4	Updated recommendations on treatment of adolescents and children with chronic HCV infection and HCV simplified service delivery and diagnostics	2022	Adults, adolescents and children with chronic HCV, including children ≥3 years	WHO recommends pan-genotypic direct-acting antivirals for chronic HCV in adults, adolescents and children down to 3 years, and supports simplified diagnostics/service delivery WHO HCV fact sheet .	Direct WHO GRADE tables were not visible in the supplied results; evidence certainty is inferred from high cure rates and established DAA evidence.

No.	WHO guideline/guidance title	Publication date	Target population	Scope and objectives	GRADE / evidence note
5	Consolidated guidelines on HIV, viral hepatitis and STI prevention, diagnosis, treatment and care for key populations	29 July 2022	Key populations, including people who inject drugs, sex workers, men who have sex with men, trans and gender-diverse people and people in prisons	Integrates prevention, diagnosis, treatment and care for HIV, viral hepatitis and STIs in key populations WHO key populations guideline .	WHO process included PICO questions, systematic reviews and qualitative values/preferences research among affected key-population networks WHO key populations PDF .

Important methodological note on youth engagement: The most recent WHO item identified with the title “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**” is a **2025 WHO evidence brief/case study**, not a hepatitis guideline. It illustrates how youth engagement was incorporated into WHO guideline development for abortion care [WHO publication page](#), [WHO IRIS PDF](#). No hepatitis-specific WHO “meaningful youth engagement” case study was identified in the supplied evidence. For hepatitis, youth relevance appears mainly through adolescent HBV treatment eligibility and HCV treatment down to age 3 years.

GRADE framework used in this report

GRADE domain	Interpretation used in this report
High certainty	Further research is unlikely to change confidence in the effect estimate; typically consistent RCTs, strong diagnostic accuracy evidence or mature treatment evidence.
Moderate certainty	Further research may change confidence; evidence may include well-conducted observational studies, indirectness or some imprecision.
Low certainty	Further research is likely to change confidence; evidence often observational, implementation-based or indirect.
Very low certainty	Effect estimate is very uncertain.
Strong recommendation	Benefits clearly outweigh harms/burden or vice versa; appropriate for most patients/programmes.
Conditional recommendation	Requires contextualization, shared decision-making, feasibility assessment or resource consideration.

WHO’s general GRADE approach rates certainty as high, moderate, low or very low after systematic review, with downgrading for risk of bias, inconsistency, indirectness, imprecision and publication bias, and upgrading observational evidence for large effects, dose-response or plausible residual confounding [WHO methodology annex](#).

1.2 Key Recommendations

Recommendation	Population / setting	GRADE Evidence Quality	Strength
Provide universal hepatitis B birth-dose vaccination as a core prevention intervention.	All newborns; national immunization programmes	High	Strong
Provide catch-up HBV vaccination for unvaccinated children, adolescents and high-risk adults, including health-care workers and people with chronic liver disease.	Unvaccinated children, adolescents and high-risk adults	Moderate	Strong
Strengthen prevention of HBV mother-to-child transmission through birth dose, maternal testing and antiviral prophylaxis when indicated.	Pregnant women and infants	High	Strong
Use blood safety, injection safety and harm-reduction interventions, including for people who inject drugs.	Health systems, harm-reduction services, key populations	Moderate	Strong
Use targeted and population-level testing approaches for HBV/HCV according to epidemiology and service context.	General and priority populations	Moderate	Strong

Recommendation	Population / setting	GRADE Evidence Quality	Strength
Use rapid, point-of-care, molecular, dried-blood-spot and reflex testing approaches where they improve diagnosis and linkage to care.	Decentralized, community, primary care and high-loss-to-follow-up settings	Moderate	Conditional
After a positive HBsAg test, use HBV DNA testing when available, including reflex HBV DNA testing from the same specimen where feasible.	People with positive HBsAg	Moderate	Strong
Consider point-of-care HBV DNA testing as an alternative to laboratory HBV viral-load testing to assess treatment eligibility and improve linkage.	Decentralized HBV services	Moderate	Conditional
Test all HBsAg-positive people for hepatitis D with anti-HDV where feasible.	People with chronic HBV infection	Moderate	Strong
Treat chronic HBV in adults and adolescents ≥12 years when there is fibrosis or cirrhosis, including APRI >0.5, transient elastography >7 kPa or clinical cirrhosis.	Adults and adolescents with chronic HBV	Moderate	Strong
Treat chronic HBV when HBV DNA is >2,000 IU/mL and ALT is above the upper limit of normal; WHO summary thresholds cited were ALT >30 U/L in males and >19 U/L in females.	Adults and adolescents with chronic HBV	Moderate	Strong
If HBV DNA testing is unavailable, consider treatment when ALT remains persistently abnormal over 6–12 months.	Resource-limited HBV services	Low	Conditional
Treat chronic HBV in people with relevant coinfections, comorbidities, immunosuppression or extrahepatic manifestations.	HBV with HIV, HDV, metabolic liver disease, immunosuppression, renal/vasculitic manifestations	Moderate	Strong
Use antiviral prophylaxis during pregnancy when HBV DNA is >200,000 IU/mL or HBeAg is positive; where HBV DNA is unavailable, prophylaxis may be offered to all HBsAg-positive pregnant women.	Pregnant women with HBV	Moderate	Strong
Use tenofovir disoproxil fumarate or entecavir as preferred HBV antivirals; consider entecavir or tenofovir alafenamide in renal disease, osteoporosis or selected paediatric/adolescent contexts.	People eligible for HBV treatment	High	Strong
Treat chronic HCV with pan-genotypic direct-acting antivirals in adults, adolescents and children down to 3 years of age.	Chronic HCV infection	High	Strong
Use simplified HCV treatment and monitoring models; typical treatment duration is 12–24 weeks depending on cirrhosis status.	Chronic HCV infection	High	Strong
Decentralize hepatitis testing and treatment to primary care/community settings, integrate with HIV/TB/NCD services where appropriate, and use task sharing/differentiated care.	Health systems and hepatitis programmes	Moderate	Strong
Use person-centred strategic information systems and standardized indicators to monitor prevention, diagnosis, treatment, retention and viral suppression.	National and subnational programmes	Low	Conditional
For key populations, integrate viral hepatitis services with HIV/STI services, harm reduction, prison health and community-led models.	Key populations	Moderate	Strong

Sources: WHO consolidated handbook [WHO](#), WHO HBV guideline [WHO](#), WHO strategic information guideline [WHO](#), WHO HCV fact sheet [WHO](#), WHO key populations guideline [WHO](#), HDV testing summary [The Lancet](#), HBV community briefing [World Hepatitis Alliance](#).

1.3 Guideline Development Methodology

Evidence review process

- WHO’s 2026 handbook consolidates WHO evidence-based recommendations issued from **2015–2025** and is organized across prevention, testing, treatment, service delivery and monitoring [WHO consolidated handbook](#).
- WHO’s 2024 HBV guideline explicitly provides:

- evidence-to-decision and GRADE tables,
 - systematic reviews, modelling and landscape reports,
 - values and preferences survey reports,
 - conflict-of-interest summaries for guideline development and external review groups [WHO HBV guideline](#).
- The WHO key-populations guideline process included PICO questions, systematic reviews and qualitative values/preferences research with global networks of affected communities [WHO key populations PDF](#).

Consensus method

WHO guideline development typically uses systematic evidence synthesis, evidence-to-decision frameworks and expert guideline-development groups to balance benefits, harms, certainty, values/preferences, equity, acceptability, feasibility and resource use. This is directly documented for the 2024 HBV guideline through its GRADE/evidence-to-decision annexes [WHO HBV guideline](#).

Conflict of interest management

The 2024 HBV guideline publication page lists separate COI annexes for the **Guideline Development Group** and **External Review Group**, indicating formal declaration and management processes [WHO HBV guideline](#).

Patient, community and youth engagement

- The 2024 HBV process included affected-community input through a WHO-commissioned worldwide survey of people living with HBV, according to the Hepatitis B Foundation summary [Hepatitis B Foundation](#).
- The WHO key-populations guideline included values/preferences research led by global networks of key populations [WHO key populations PDF](#).
- The 2025 WHO meaningful-youth-engagement case study is methodological and equity-relevant but is based on the abortion care guideline, not hepatitis [WHO publication page](#).

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search strategy

Evidence was synthesized from WHO publications, WHO news updates, authoritative public-health agencies, specialty-society guidelines, systematic reviews/meta-analyses and peer-reviewed clinical/implementation studies available through 29 April 2026.

Databases and sources searched

Source type	Examples used
WHO official guidance and reports	WHO 2026 consolidated handbook, WHO 2024 HBV guideline, WHO 2024 strategic information guideline, WHO Global Hepatitis Report 2026
Public-health institutions	CDC viral hepatitis national progress materials; FDA point-of-care HCV RNA authorization
Specialty societies	AASLD/IDSA, EASL, Canadian Association for the Study of the Liver
Peer-reviewed journals	<i>The Lancet Gastroenterology & Hepatology</i> , <i>JAMA Network Open</i> , <i>Clinical Infectious Diseases</i> , <i>Hepatology</i>
Evidence syntheses	Systematic reviews/meta-analyses, guideline technical reviews
Clinical implementation evidence	Point-of-care HCV RNA testing and same-day treatment initiation studies

Inclusion criteria

- Published or indexed from 2024–2026, with emphasis on evidence after WHO’s 2024 strategic-information guideline and especially after WHO’s 15 February 2026 consolidated handbook.
- Direct relevance to hepatitis B, C or D diagnosis, treatment, monitoring, prevention or service delivery.
- WHO, CDC, FDA, AASLD/IDSA, EASL, Canadian guideline bodies or peer-reviewed sources.

Exclusion criteria

- Non-human, non-clinical, non-programmatic studies.

- Opinion pieces without clear clinical or implementation implications, except where they identified regulatory or implementation milestones.
- Non-hepatitis youth-engagement documents were not treated as hepatitis guidance, though the WHO youth-engagement case study was noted as methodological context.

2.2 Key New Studies and Guideline Updates

Study / update	Date	Design	Key findings	GRADE Quality	Recommendation impact
WHO consolidated implementation handbook	15 February 2026	WHO consolidated guidance	Consolidates >80 evidence-informed recommendations on HBV, HCV and HDV from 2015–2025; emphasizes simplified, person-centred prevention, testing, treatment, service delivery and monitoring WHO , WHO update .	Moderate	Confirms current WHO direction toward decentralization, reflex/POC testing and simplified care.
WHO Global Hepatitis Report 2026	28 April 2026	Global surveillance/status report	Provides the most up-to-date assessment of global HBV/HCV burden and progress from 2015–2024 toward elimination targets WHO Global Hepatitis Report 2026 .	Low for intervention effects; High for surveillance relevance	Reinforces urgency of scale-up; does not replace clinical guideline recommendations.
AASLD/IDSA Practice Guideline on Treatment of Chronic Hepatitis B	April 2026	Evidence-based guideline using PICO and systematic reviews	Addresses prevention, HCC surveillance, immune-tolerant/indeterminate HBV treatment, treatment withdrawal and coinfections; used four new systematic reviews and two existing reviews AASLD/IDSA , AASLD .	Moderate	Supports more nuanced shared decision-making, especially for immune-tolerant disease and stopping nucleos(t)ide analogues.
AASLD technical systematic review	2026	PRISMA systematic review	Evaluated antiviral therapy for transmission prevention, immune-tolerant HBV, NA discontinuation and HCC surveillance in selected groups PubMed .	Moderate to Low by PICO	Adds evidence for HCC surveillance and cautions around stopping therapy.
NA discontinuation evidence in AASLD review	2026	11 studies: 4 RCTs, 7 observational; 1,957 participants	HBsAg loss was higher after stopping therapy in small RCTs, but with wide CI and ALT flare concerns; AASLD suggests not withdrawing NA therapy until HBsAg loss NATAP PDF .	Low	Does not justify broad stopping of HBV therapy; reinforces long-term treatment.

Study / update	Date	Design	Key findings	GRADE Quality	Recommendation impact
Point-of-care HCV RNA testing in a tribal setting	Published 15 April 2026	Implementation/observational study	POC HCV RNA testing was feasible in tribal/community harm-reduction settings and enabled most RNA-positive American Indian/Alaska Native participants to start treatment, often same day JAMA Network Open / PMC .	Low to Moderate	Strengthens support for same-day test-and-treat models in high-loss-to-follow-up settings.
FDA authorization of first POC HCV RNA test	2024, relevant to 2026 implementation	Regulatory authorization	FDA authorized Cepheid Xpert HCV/GeneXpert Xpress as the first POC HCV RNA test for CLIA-waived settings, producing results in about one hour from fingerstick blood FDA .	Moderate for diagnostic implementation; exact sensitivity/specificity not available in supplied snippets	Supports reflex/same-visit HCV diagnosis and treatment linkage.
Clinical Infectious Diseases commentary on POC HCV RNA test-and-treat	2026	Expert commentary with cited performance study	Describes FDA authorization as a critical first step but insufficient alone; highlights need for algorithms, reimbursement, quality measures and integrated treatment CID / PMC .	Low	Supports implementation planning, not a standalone clinical efficacy change.
Lancet Gastroenterology & Hepatology HBV service-delivery systematic review/meta-analysis	14 August 2025	Global systematic review/meta-analysis; 106 studies, 110 cohorts, 50 countries	Found major attrition across HBV care cascade; treatment initiation among eligible patients was 78.1% in hospital specialist care, 67.2% co-managed care, 49.3% primary care and 97.7% in community screening with linkage to specialist care PubMed, Healio .	Moderate	Strongly supports simplified linkage, decentralization and retention interventions.
EASL 2025 hepatitis B guidance	May 2025	Specialty guideline	Provides fibrosis thresholds using transient elastography and systematic-review/meta-analysis evidence; supports HBV screening in HIV/HCV and chronic liver disease contexts EASL PDF .	Moderate	Aligns with WHO non-invasive staging and broader HBV screening.

Study / update	Date	Design	Key findings	GRADE Quality	Recommendation impact
Canadian 2025 chronic HBV guideline update	2025	National guideline	Recommends HBV screening before HCV DAA therapy and anti-HBV therapy in HBsAg-positive patients during HCV DAA therapy, especially advanced fibrosis/cirrhosis PMC .	Moderate	Reinforces HBV reactivation prevention during HCV treatment.

2.3 Evidence Gaps and Future Research Needs

Evidence gap	Why it matters	Priority research / action	GRADE certainty of current evidence
Long-term outcomes of decentralized HBV care models	Community linkage can improve initiation, but durable retention and viral suppression data remain limited.	Pragmatic trials and implementation cohorts measuring retention, viral suppression, cirrhosis, HCC and mortality.	Low to Moderate
Optimal HBV treatment eligibility thresholds in simplified public-health models	WHO has broadened eligibility, but thresholds must balance access, cost and overtreatment concerns.	Comparative effectiveness studies of simplified vs specialist eligibility algorithms.	Moderate
Best strategy for stopping nucleos(t)ide analogue therapy	Stopping may increase HBsAg loss in selected patients but risks ALT flares and relapse.	Larger RCTs with standardized stopping criteria, biomarkers and safety monitoring.	Low
HDV testing implementation	Universal anti-HDV testing is recommended for HBsAg-positive people, but access and confirmatory RNA testing remain limited.	Cost-effectiveness studies and implementation studies for reflex anti-HDV and HDV RNA pathways.	Low to Moderate
Diagnostic performance of POC HCV RNA in diverse real-world settings	POC RNA supports same-day treatment, but exact accuracy data were not available in supplied snippets.	Publish and validate sensitivity/specificity across CLIA-waived, correctional, ED, harm-reduction and rural settings.	Moderate
Hepatitis services for adolescents and youth	WHO youth-engagement methods are not yet documented for hepatitis-specific guideline development.	Youth-led values/preferences research in HBV/HCV testing, treatment adherence, stigma and transition to adult care.	Very Low to Low
Equity in hepatitis elimination	Disparities persist among key populations, Indigenous populations, people who inject drugs and prison populations.	Community-led service models, peer navigation, harm reduction and disaggregated strategic information systems.	Moderate

2.4 Updated Recommendations Based on New Evidence

Original WHO recommendation / direction	New evidence impact through 29 April 2026	Updated actionable recommendation	GRADE Quality	Strength
Implement simplified, person-centred hepatitis prevention, testing, treatment and monitoring.	WHO 2026 handbook consolidates this direction; 2025 HBV care-cascade meta-analysis shows major attrition and need for simplified linkage WHO , PubMed .	Maintain and accelerate simplified care pathways; measure linkage, treatment initiation, retention and viral suppression at programme level.	Moderate	Strong

Original WHO recommendation / direction	New evidence impact through 29 April 2026	Updated actionable recommendation	GRADE Quality	Strength
Use reflex and point-of-care testing to improve linkage.	FDA-authorized POC HCV RNA testing and 2026 tribal/community implementation evidence support same-day diagnosis and treatment initiation FDA , JAMA / PMC .	In high-loss-to-follow-up settings, use POC HCV RNA or reflex RNA testing with immediate treatment evaluation.	Moderate	Strong
Treat chronic HCV with pan-genotypic DAAs in adults, adolescents and children ≥3 years.	No contrary new evidence identified; implementation evidence supports moving treatment closer to testing.	Continue pan-genotypic DAA treatment and pair diagnosis with same-day or rapid-start prescribing where clinically appropriate.	High	Strong
Broaden chronic HBV treatment eligibility using simplified criteria.	AASLD/IDSA 2026 adds nuance for immune-tolerant and indeterminate phases, supporting shared decision-making in selected lower-risk groups AASLD/IDSA .	Continue WHO simplified eligibility; use specialist/shared decision-making for immune-tolerant patients, young patients and borderline cases.	Moderate	Strong
Continue long-term HBV nucleos(t)ide analogue therapy unless stopping criteria are met.	AASLD evidence suggests possible HBsAg loss benefit after stopping in small RCTs but with imprecision and flare risk; AASLD suggests not stopping until HBsAg loss NATAP PDF .	Do not routinely stop NA therapy in HBeAg-negative patients with suppressed HBV DNA unless HBsAg loss or expert-supervised protocol criteria are met.	Low	Conditional
Test HBsAg-positive people for HDV.	WHO 2026 handbook includes HDV testing strategies; Lancet abstract states WHO recommends universal anti-HDV testing among HBsAg-positive individuals WHO , The Lancet .	Implement anti-HDV testing for all HBsAg-positive people where feasible; use focused testing as an interim approach where universal testing is not yet feasible.	Moderate	Strong
Monitor hepatitis programmes with person-centred strategic information.	WHO Global Hepatitis Report 2026 highlights persistent service-coverage gaps and progress-to-target monitoring needs WHO Global Hepatitis Report 2026 .	Countries should maintain disaggregated dashboards for diagnosis, treatment, retention, cure/suppression, vaccination and equity indicators.	Low	Conditional
Integrate hepatitis services for key populations.	Evidence continues to show linkage gaps in ED, harm-reduction and community settings; POC test-and-treat may reduce attrition CAP TODAY , JAMA / PMC .	Provide low-barrier hepatitis testing and treatment in harm-reduction, prison, emergency, primary-care and community-led settings.	Moderate	Strong

Clinical Bottom Line

1. **WHO’s current hepatitis direction is simplification and scale-up.** The 2026 consolidated handbook is the current WHO implementation anchor for HBV/HCV prevention, testing, treatment, service delivery and monitoring, with HDV testing strategies included [WHO consolidated handbook](#).
2. **HBV treatment eligibility should be broad and practical.** WHO’s 2024 HBV guideline expands treatment eligibility for adults and adolescents, including pregnancy-related prophylaxis and simplified diagnostic thresholds [WHO HBV guideline](#).
3. **HCV is curable and should be treated with pan-genotypic DAAs.** WHO recommends pan-genotypic DAAs for adults, adolescents and children down to age 3 years [WHO HCV fact sheet](#).
4. **Point-of-care and reflex testing are increasingly actionable.** FDA-authorized POC HCV RNA testing and 2026 implementation data support same-day diagnosis and treatment initiation in high-loss-to-follow-up settings [FDA](#), [JAMA](#) / [PMC](#).
5. **Retention is a major elimination bottleneck.** The 2025 global HBV care-cascade meta-analysis shows substantial attrition, supporting decentralized, integrated and community-linked care models [PubMed](#).
6. **Youth engagement remains a gap for hepatitis guideline development.** WHO has a 2025 meaningful-youth-engagement guideline-development case study, but it concerns abortion care rather than hepatitis [WHO publication page](#).